

# ■ Pain Relief in Labor

Your options for managing labor pain

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Every labor is different — and so is every person's experience of pain. The good news is that you have many options, and you don't have to decide everything in advance. This pamphlet outlines your choices so you can think about what feels right for you and discuss it with your care team.

## NON-MEDICATION OPTIONS

These techniques can significantly reduce pain and are safe at any point in labor. Many people use them alongside medication.

### Movement & Position

- Walking, rocking, or swaying during contractions
- Hands-and-knees position relieves back labor
- Birthing ball for hip movement and pressure relief

### Touch & Heat

- Massage and counterpressure on the lower back
- Warm compress or shower/tub (hydrotherapy)
- Cold pack on the lower back between contractions

### Breathing & Relaxation

- Slow, focused breathing through each contraction
- Visualization — imagining a calm, safe place
- Guided relaxation or meditation apps

### Support

- Continuous support from a partner, doula, or nurse significantly reduces pain perception
- Calm environment — dim lights, familiar music, or quiet

## MEDICATION OPTIONS

### ■ Epidural Anesthesia

The most commonly used and most effective form of pain relief in labor. A small flexible catheter is placed in your lower back by an anesthesiologist. Medication flows continuously to numb the lower half of your body.

- Very effective pain relief
- You stay awake and aware
- Can be adjusted anytime

- May lower blood pressure
- May slow pushing phase
- Limits mobility

### ■ IV / Injectable Opioids

Medications such as fentanyl or morphine can be given through an IV or by injection to take the edge off contractions. They don't eliminate pain completely but can help you rest between contractions.

- Short-acting — effects wear off in 1–2 hours
- May cause drowsiness, nausea, or dizziness
- Timing matters — avoided close to delivery to prevent newborn breathing effects

### ■ Local Anesthesia (Perineal Block)

An injection directly into the perineal tissue, used at the time of delivery to numb the area for repair of a tear or episiotomy. It does not reduce contraction pain.

## YOUR BIRTH PLAN & PAIN PREFERENCES

**You can change your mind at any time.** Choosing or declining an epidural early does not lock you in. Many people who plan unmedicated births choose an epidural once active labor begins — and that is completely fine. Likewise, an epidural can be given at almost any point before delivery.

**■ Talk to us before your due date about your preferences.** We will share your birth preferences with the Labor & Delivery team. The goal is a safe, supported experience — whatever that looks like for you.

